PROJECT OVERVIEW

AI-Native Compliance Infrastructure

Regulatory-change intelligence for Medicaid payers and state agencies

1. The Objective (What We're Building)

An AI tool that watches Medicaid/payer regulations and automatically tells a compliance team: “this rule changed, here’s exactly what internal policy/process it affects, and here’s a draft of the update.” Think of it as a compliance analyst that never sleeps and never misses a CMS bulletin.

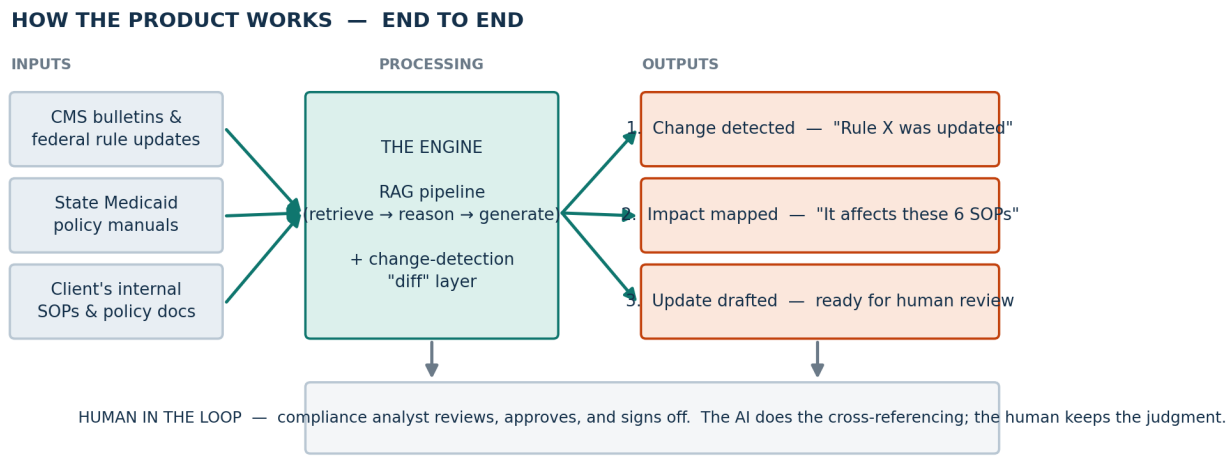


Figure 1 — The product pipeline: public and internal documents go in, detected changes and drafted updates come out, with a human approving every result.

2. The Core Problem We're Solving

State Medicaid agencies and MCOs (managed care organizations) currently track regulatory changes manually — someone reads CMS/state bulletins, cross-references them against internal policy manuals, and updates SOPs by hand. This is slow, inconsistent across teams, and the stakes are high: regulators are holding payers and providers accountable for billing practices with growing enforcement pressure, and CMS is using AI to preemptively flag fraud. If your compliance tracking lags, you’re exposed to audits, clawbacks, or CMS penalties.

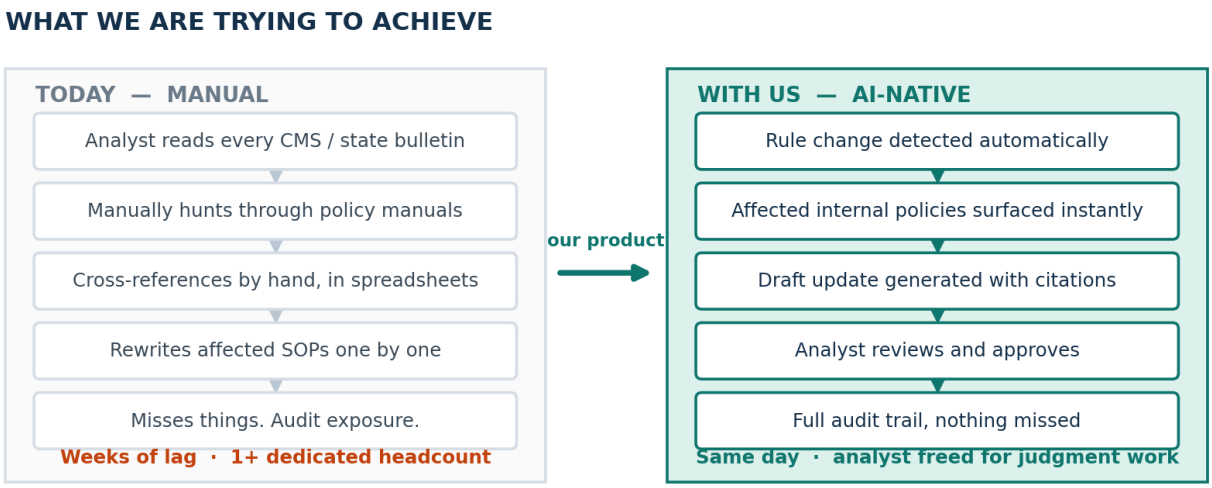


Figure 2 — The shift we are selling: from weeks of manual cross-referencing to same-day detection with the analyst kept in the loop.

3. Who Pays You

- **State Medicaid agencies** (like where you work) — but selling back to your own employer is a conflict of interest, so treat this as "other states," not Alabama
- **MCOs** (managed care orgs contracted by states — UnitedHealthcare Community Plan, Centene, Molina, etc.)
- **Compliance/RCM (revenue cycle management) consulting firms** who serve payers and want to offer this as a tool

4. Is There Real Demand?

Yes — and it's accelerating, not shrinking. 2026 marked a turning point where healthcare AI moved from a "wild west" into a structured, increasingly enforceable regulatory environment across both federal and state jurisdictions. CMS has already rolled out specific AI guardrails for prior-authorization processes at Medicare Advantage plans, and the Joint Commission and CHAI are preparing a voluntary AI certification program for 2026 — meaning payers will soon need to *prove* their compliance processes meet a bar, not just claim they do. That's a forcing function that creates budget for tools like yours.

5. Is It a Monopoly? (Be honest here)

No — and don't tell investors it is; that kills credibility. The space is getting crowded on the *edges*:

- Clair Health already does AI-driven revenue cycle management for billing companies, EHRs, and provider groups
- Ritivel does AI-native regulatory writing for life sciences/pharma companies drafting FDA submissions
- General healthcare AI vendors build compliance-first architecture for payers with HIPAA/SOC2/HITRUST certifications already baked in

Nobody appears to own the specific niche you'd own: real-time regulatory-change tracking specifically for *state Medicaid rule updates*, mapped to internal payer policy. That's your wedge — narrow enough to be defensible, but you're not creating a category from nothing.

6. How You're Different

- **Domain-first, not tech-first**: most competitors are engineering shops that bolted AI onto generic compliance software. You *are* the domain expert — 5+ years inside actual Medicaid operations. That's your credibility moat.
- **State-specific granularity**: Medicaid rules vary by state. A tool that's actually good at tracking *state-level* nuance (not just federal CMS) is harder to build and more valuable to MCOs operating across multiple states.
- **Narrow wedge, not a platform**: you're not trying to be an all-purpose compliance suite (that's the graveyard most of these startups end up in) — you start with one sharp use case and expand.

7. Cost to the Client (rough starting model)

- **SaaS subscription**, not per-seat — payers hate headcount-based pricing for compliance tools
- Ballpark for a state-level MCO: **$3,000–$8,000/month**, depending on how many regulatory sources and how many internal policy documents it needs to track
- Cheaper than the cost of one specialized compliance analyst salary (~$70–90k/year), which is your pitch: "we replace the manual cross-referencing work, not the judgment"

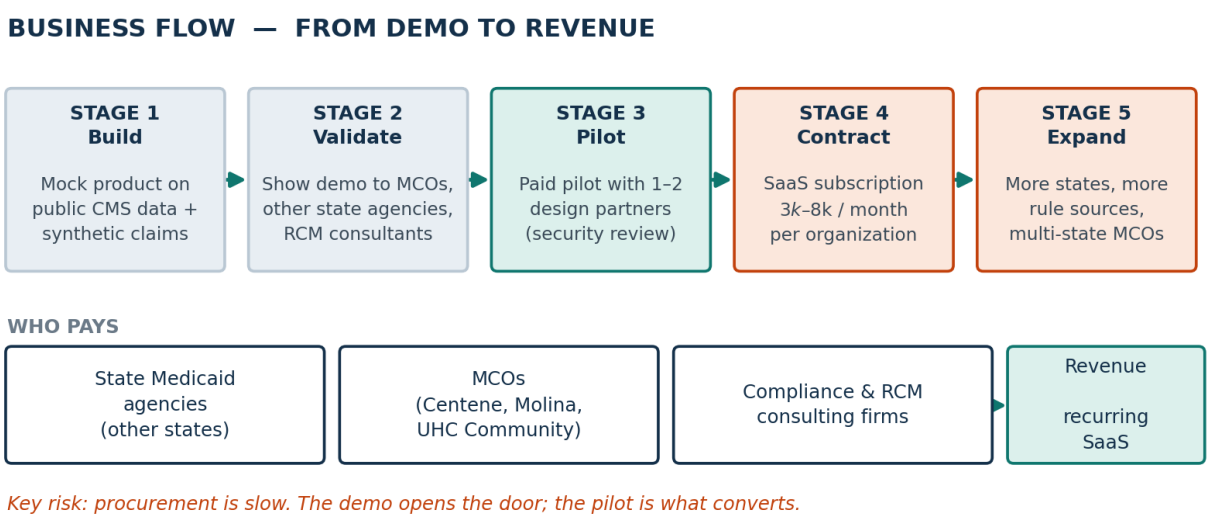


Figure 3 — Business flow: how the mock demo turns into pilots, contracts, and recurring revenue, and who writes the cheque at each stage.

8. Is It Efficient / Workable to Build?

Yes, and this is your advantage: you already have the working engine (Healthcare Claims Translator AI project — RAG pipeline on Ollama/Qwen3/Gemma). The remaining work is:

- Swap in *public* regulatory sources instead of any employer data
- Build a clean ingestion pipeline for policy documents (PDF parsing)
- Add a “diff” layer — comparing old vs. new regulation text to flag what changed
- Wrap it in a presentable UI

None of this requires new invention — it’s assembly of proven techniques, which is exactly the kind of MVP a non-technical-heavy team (you + maybe one engineer) can ship in weeks, not years.

9. Biggest Risk

Not technical — it’s **sales cycle length**. Government/payer procurement is slow (RFPs, security reviews, procurement committees). Your mock demo won’t close a deal in one meeting; it’s a foot-in-the-door to get a pilot conversation started, likely months before revenue.